CONSULT FOR AFRICA

Arabella Women's Health

Diagnostic Audit: Interim Report

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For Dr Chito Nwana, Arabella Women's Health
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A letter before the report

Dr Chito, thank you for how much you and Tolu have sent us in a short time. Twenty four uploads, statements from two banks, and two thoughtful leadership answers in three weeks is more than most clients manage.

When we started you asked for three things in your own words: what needs immediate attention, what can wait, and what you should strengthen before expanding further. This report answers those three questions with the evidence we have. It is an interim report because some of the evidence is still missing, and section 7 says exactly what, and which conclusions would change when it arrives.

A few things I want to say plainly up front.

The first is that Arabella is a sound practice. Your team rates patient safety highly, your laboratory is properly run and properly licensed, your patient list is real and large, and the staff who answered us like working for you and like how patients are treated. That is the foundation, and none of what follows takes away from it.

The second is that the plan you were given leads on maternity, surgery and fertility, and the records show those are not yet where the business earns. Today Arabella earns from its laboratory and its consultations. I do not think that is a reason to abandon the plan. I think it is a reason to fix conversion and cost first, and expand second.

The third is the facility registration. The only registration we have seen is for Tabitha at Jabi, and the 2026 renewal was paid but has not yet been granted. I would like to put this right with you in the next fortnight, before it becomes anyone else's question.

The natural next step is a one hour session with you and Tolu to go through section 2, and to agree who closes each of the gaps in section 7. I will make the time whenever suits you.

Warm regards,

Dr Debo Odulana

1. The answer in one page

- Revenue: N48.7m taken January to September 2026, about N5.4m a month, from about 26 paying patients a month. You and Tolu both put it at N5m to N8m, so the baseline is agreed.
- Where it comes from: laboratory 30%, consultations 22%, fertility 12%. Maternity packages are 6%.
- Constraint: conversion and cost. Patients arrive and do not stay: 7 in 10 pay once, and 7 of the year's 11 maternity bookings delivered elsewhere.
- Cost: on our reading of the expense sheets, January to July operating cost of about N40.0m ran slightly above revenue of N37.4m, while N24.8m went into the Asokoro renovation.
- Compliance: the Asokoro premises needs its own facility registration, the 2026 renewal needs to be granted, and pharmacy sales need a pharmacist and a premises licence.
- People: a team that is proud of its care and wants more trust to decide. Everything runs through you and the Head of Nursing, which you, Tolu and your staff each named on their own.
- Recommendation: put the registration right now, fix conversion and cost over the next quarter, and expand into maternity, surgery and fertility only once those hold.

2. Your three questions

Needs immediate attention

- Facility registration: register the Asokoro premises in Arabella's name, and get the 2026 renewal decision in writing. Detail in section 5.
- Pharmacy: appoint a superintendent pharmacist and apply for the pharmacy premises licence, or stop dispensing until both are in place.
- Cash: neither First Bank account held a working balance on 2 October. Set a minimum cash floor and a weekly cash report before any further capital spend.
- Front desk confidentiality: patient cases are discussed where waiting patients can hear. Move those conversations out of the waiting area this week.

Strengthen before expanding

- Maternity conversion: find out why 7 of 11 booked mothers delivered elsewhere, and fix that before marketing more packages.
- Follow-up: book the next appointment before the patient leaves. 72% of patients paid once this year.
- Cost base: two salary rolls ran together from July. Merge the Tabitha and Arabella cost base into one, with one budget.
- Delegation: give named people authority to decide, with clear limits, so that the practice does not wait for you.
- Insurer income: chase what HMOs owe and put each panel on a current tariff. Insurers paid under 2% of receipts.
- Patient contact data: capture a mobile number at every visit. Only 14% of records have one.

Can wait

- Building an in-house fertility unit. Today fertility is referred to a partner laboratory at a 42% margin. Keep that until volume justifies a unit.
- Expanding surgery and large-scale marketing. Both depend on conversion and registration being settled first.
- Contacting the full patient list for marketing. First tell patients about the change of name and give them an easy way to opt out.

3. What Arabella earns

Measure	Figure	Source
Taken, January to September 2026	N48.7m (285 payments)	Sales record
Average month	N5.4m (N4.7m without the largest single payment)	Sales record
Paying patients a month	About 26	Sales record
Patients who paid only once	141 of 196 (72%)	Sales record
July and August	N2.3m and N1.8m	Sales record

Service line	January to September	Share
Laboratory	N14.6m	30%
O&G and GP consultations	N10.6m	22%

Service line	January to September	Share
Fertility (referred to a partner laboratory)	N6.0m	12%
Pharmacy	N4.2m	9%
Procedures	N4.0m	8%
Maternity packages	N3.0m	6%
Other (wellness, admissions, imaging, registration, visiting consultants)	N5.5m	11%
HMO payments	N0.8m	2%

- Strong months are single cases: March includes one N6.5m payment, mostly fertility. September includes one N2.0m procedure.
- Laboratory: 33 of 43 tests are done in house and 10 are sent out. This is the line that is most fully Arabella's own.
- Fertility: cost N3.5m, charged N6.0m including donor. It is a referral with a margin, not a service Arabella runs.
- Maternity: 11 bookings in the year. 1 delivered at Arabella, 7 delivered elsewhere, 3 still in antenatal care.
- Insurers: the patient list holds 483 HMO or company registrations, yet insurers paid N0.8m in the sales record. HMO claims land in the GTB account: N1.13m January to September 2026, then N1.15m in the first 13 days of October.

4. Cost and cash

- Tabitha expenses, January to October 2026: N75.9m, of which renovation of the Asokoro premises N24.8m and salaries N23.8m.
- Operating position, January to July: cost about N40.0m against revenue of N37.4m, excluding renovation.
- Arabella expenses: N10.2m, mostly from July, including N5.9m of salaries from July to September.
- Two cost bases: from July both entities carried staff and running costs at the same time.
- Bank balances on 2 October 2026: Tabitha First Bank N701,699, Arabella First Bank N276,405. Over two years, money in and money out match almost exactly in both accounts.
- Trading credits: Tabitha ran N4.6m to N8.6m a month through 2024 and 2025, which supports the N5.4m sales record figure.
- Large non-trading flows: 2026 includes significant transfers in and out, including funds you put in personally. Until each is classified, total bank credits cannot be read as revenue. The items are listed in our email of 21 October.

5. Registration and licensing

Item	What we found	What is needed
Facility registration	Registered as Tabitha Medical Centre, Plot 35 ICS Drive, Jabi, first registered 8 December 2011, category 6-bed hospital	Registration of 37 Nelson Mandela Street, Asokoro, in Arabella's name

Item	What we found	What is needed
2026 renewal	Fee of N150,322.50 paid 21 February 2026. The official section is blank. Last granted renewal: March 2025	Written decision from the registration committee
Declared establishment	3 doctors, 3 nurses, 1 laboratory scientist, no pharmacist	An establishment that matches the services now offered
Pharmacy	Pharmacy is a revenue line (N4.2m) with stock on hand	Superintendent pharmacist and premises licence
Medical licences	Dr Chito: full MDCN registration, 2026 practising licence valid to 31 December 2026. Two further doctors licensed	Nothing outstanding
Specialist of record	The pack does not name a specialist obstetrician. You may hold qualifications we were not sent	Send any specialist qualification, or name the obstetrician of record. HMOs and referrers ask, and indemnity cover should match the work done
Nursing and laboratory	Three nurses licensed to 2027 and 2028. Laboratory scientist licensed for 2026, with a Lagos address on the licence	Confirm the laboratory scientist works in Abuja
Fertility	No assisted reproduction licence, which is consistent with fertility being referred	The partner laboratory agreement
Company	Arabella Women's Health & Wellness Ltd, RC 1802696, incorporated 7 June 2021, renamed September 2024	Nothing outstanding

Each of these is fixable, and most are paperwork. The registration of the Asokoro premises is the one I would not leave, because it is the one that affects every insurer contract and every referral letter.

6. People and leadership

- Staff survey: 13 of about 25 staff answered. Average score 4.07 out of 5.
- Safety: 9 rate patient safety excellent and 3 very good. 12 of 13 would be happy for their own family to be cared for at Arabella.
- Strongest scores: sterility and sterilisation records 5.0, kindness to patients 4.8, medicines in date 4.7.
- Weakest scores: questioning a senior decision without it being held against you 3.1, being afraid to ask questions 3.0. In a maternity unit this matters most in an emergency.
- Founder dependency: you wrote that you want the business to function without everything depending on you. Tolu and a member of staff said the same independently.
- Start time: the day is meant to begin at 8am and usually begins nearer 9am.
- Your two leadership answers differ on the constraint: you see cash to invest and a 12-month target above N60m; Tolu sees patient demand and a target of N25m to N40m. The records support neither fully. They point to conversion and cost, which is the conversation to have together.

7. The gaps, and what each would change

These are the items we asked for and have not received, or have received only in part. Each one limits a specific conclusion in this report.

Gap	Status at 21 October	What it would change
Patient survey	0 responses	Why mothers deliver elsewhere and why patients pay once. Today these are measured but not explained
Referrer survey	0 responses	Where patients come from, and what referring doctors need before they send more
Access Bank statement	Not received	The full cash picture. Section 4 covers First Bank and GTB only
Reply to our 21 October email	Not received	Which 2026 bank credits are revenue, and whether funds you put in are loans or capital
Revenue split by payer	Not received	The true insurer share, and what HMOs owe
HMO contacts and remaining tariffs	Partial: names for all panels, tariffs for Leadway, AXA Mansard and Allianz, no contact names	Which panels are worth keeping, and who to chase
Asokoro facility registration and 2026 renewal decision	Not received	Whether section 5 is a paperwork fix or a live exposure
Pharmacy licence and pharmacist	Not received	Whether dispensing can continue as it is
Specialist obstetrician of record and indemnity cover	Not received	Insurer and referrer confidence in the maternity service
Partner laboratory agreement for fertility	Not received	The real margin and liability on fertility
Name-change notice to patients	Not answered	Whether the patient list can be used for marketing
Staff survey coverage	13 of about 25	Section 6 reflects half the team

All of these can be uploaded at consultforafrica.com/ArabellaProject. The two surveys take a patient about four minutes and can be sent from the front desk by WhatsApp.

8. Next steps

1. This week: a one hour session with you and Tolu on section 2.
2. Within two weeks: the registration and pharmacy steps in section 5 started, with copies uploaded.
3. Within two weeks: the patient survey sent to every patient seen since January, and the referrer survey to your ten main referrers.
4. On receipt of the gaps: the final diagnostic report, with the maternity and cash conclusions confirmed or corrected.

Appendix: what this report is built on

- Sales record, January to September 2026, 285 payments

- Expense sheets for Tabitha (January to October 2026) and Arabella (from 2025)
- Bank statements: First Bank (Tabitha and Arabella, October 2024 to 2 October 2026), GTB (Tabitha, October 2024 to 13 October 2026)
- Price list, staff list, HMO list, three tariffs, three payer contracts, pharmacy stock
- Service breakdowns for maternity, fertility and the laboratory
- Registration pack, company certificate and professional licences
- Patient directory export, 4,379 records, counted only
- Staff survey (13 responses) and leadership survey (Dr Chito Nwana and Tolu Williams)

Your dashboard in the client portal shows the revenue, patient and survey figures in more detail.